



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمه محمد انور محمد احمد
المحافظة : القاهرة الحي / المدينة : المطرية
رقم البطاقة : 28311250107804 رقم التليفون : 01279428200
العنوان : ++++++
العمر (تاريخ الميلاد) : 25-11-1983 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 03-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني تاموكسفين للزولاديكس قسم العلاج :
تحت الجلد (6 اشهر) [14164]
الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فاطمه محمد انور محمد احمد National ID: 28311250107804	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي (Zoladex plus Goserelin Tamoxifen)

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

فاطمه
محمد انور محمد احمد
٩ ش وابور الطحين
الخصوص - القليوبية

١٩٨٣/١/٢٥ ٢٨٣ ١١ ٢٥ ٠١ ٠٧٨ ٠٤
JN4823797

٢٠٢٢/٠٨ ٢٨٣ ١١ ٢٥ ٠١ ٠٧٨ ٠٤
حاصله على دبلوم تجاره
أنثى مسلمة
البطاقة سارية حتى ٢٠٢٩/٠٨/٢٢ ١١

١١



Patient Name	: الأستاذة / فاطمة محمد انور محمد احمد	Request No.	: 135464
Patient No.	: 28311250107804	Request Date	: 20/09/2021
Path No.	: SR-461	Verification Date	: 25/09/2021
Sex	: Female	Financial Category	: ام دار السلام - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referring Doc :			

Pathology Report

Clinical Data: Right breast mass (as per the involved report)

Nature of the specimen: Slide revision

Gross Examination: 1 blocks labeled (6524) and 3 slides labeled by the patient name).

- Procedure: SLIDE REVISION
- Localization: RIGHT
- Immunohistochemistry technique: N/A

Microscopic examination:

- Histological type: invasive duct carcinoma with micropapillary features.
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II
- Angioinvasion: No
- Ductal carcinoma in situ: No
- Additional pathologic findings: No

Diagnosis:

Right breast mass, Slide revision

- Invasive duct carcinoma, grade II, with micropapillary features.
- Biomarkers:
 - o ER: Strong 90% nuclear reaction (Allred score 7/8)
 - o PR: Moderate 20% nuclear reaction (Allred score 4/8)
 - o Her2: Strong >10% membrane reaction (score 3+)

Prof. Dr. Manal ElMahd
Dr.Mona Mohamed

Printing Date : 25/09/2021 11:06:13

Page 1 of 1

Printing By : mona mohamed

Name: فاطمة محمد انور MR. : 1933	
Detailed Medical Report	
السيد	
تحية طيبة وبعد,,	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient was diagnosed in 2021 with stage T2N0M0 invasive ductal carcinoma, grade 2, triple-positive breast cancer. Neoadjuvant chemotherapy with doxorubicin and cyclophosphamide followed by paclitaxel and trastuzumab was administered, after which the patient underwent radiation therapy and a modified radical mastectomy achieving pathologic complete response. Post-operative therapy included trastuzumab for one year, followed by hormonal therapy with tamoxifen and zoledex (zoledronic acid) initiated in April 2022 and continued to the present; tamoxifen has been taken for three months and zoledex has been administered without adverse events as of February 2026. The patient continues on zoledex and tamoxifen for an additional three months, with the next clinic visit scheduled for 1 September 2026. The patient received zoledronic acid (Zometa) and calcium gluconate 2 ampules diluted in 250 mL of 5 % dextrose, infused over one hour. A consultation was performed regarding the plan for hormonal therapy after completion of a five-year course in March 2027; the same regimen of zoledronic acid and calcium gluconate was reviewed. A transvaginal ultrasound was ordered; the patient was instructed to continue zoledex and tamoxifen for three months. Follow-up for an adenoma is planned after six months, with the next clinic visit scheduled for 26 November 2026.	
تحريراً في ,	
التاريخ :	18-09-2026
الطبيب المعالج :	نهى سعيد علي محمد
يعتمد ,	



Patient Name : الأستاذة / فاطمة محمد انور محمد احمد	Request No. : 13008675
Patient No. : 28311250107804	Request Date : 05/03/2022
Path No. : 618/2022	Verification Date : 19/03/2022
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 38 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Rt breast cancer (Post-Chemotherapy)

Nature of the specimen: Simple mastectomy + SLN

Gross Examination:

- Procedure: Simple mastectomy measured 13x12x1.5cm with covering skin measured 16x5cm
- Localization: Rt
- Tumor bed site extent
 - o Number: one firm fibrotic area
 - o Size: 2x1cm
 - o Skin involvement: Free grossly
- Lymph nodes number and size: 3 nodules up to 1.8x1cm, with focal fatty infiltration
- Frozen section: Free (0/3) SLNs

Microscopic examination:

- No residual tumour cells in tumour bed
- o Skin involvement: Free nipple and areola
- Lymph node involvement: Free (0/3) SLNs
- Resection Margins:
 - o Free
- Histologic response to therapy: Pathological complete response

Diagnosis:

Rt breast cancer, Simple mastectomy + SLN:

- Pathological complete response
- o Free nipple and areola
- Free (0/3) SLNs
- Free Resection Margins

Verified by :
dr.menna atef

Printing Date : 21/03/2022 09:58:25

Page 1 of 2

Printing By : dr.Enas

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة
مركز الطب المتخصص
في أورام دار السلام
إدارة الأشعة

Patient Name: Fatma Mohamed Anwar Mohamed Ahmed	Date : 11/03/2023
Exam : MAMMOGRAPHY BOTH	Age : 39 Year(s)
Medical ID: 3663/2021	Pat. ID: 16443

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM

Technique

- left breast MLO and CC views
- Bilateral ultrasound

Report:

- The left breast scattered fibroglandular densities of the right breast (ACR B)
- Left UOQ loosely attached rounded microcalcifications measuring 10x6 mm. US showed 2-3:00 well-defined hypoechoic oval shaped solid lesion measuring g 10x6 mm
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- right MRM with free operative bed with no evidence of recurrent mass lesions
- stationary left UOQ benign looking microcalcifications with benign looking nodule (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada
11/03/2023 2:35 PM

PATIENT NAME: FATMA MOHAMMED ANWAR MOHAMMED	PATIENT ID #: 2020117424
GENDER: Female	ORDER #: 1208292,1208293
AGE: 37 y	DATE OF SERVICE: 05-Sep-2021 01:33 PM

Report: - all studies

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN AND PELVIS**

1-CHEST

- Right breast UOQ three well defined hyperdense lesions, the largest is measuring 30x19 mm.
- Tiny middle lobe pulmonary nodule.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected
- Bilateral thyroid lobes hypodense nodules fr US correlation.

2- ABDOMEN AND PELVIS

- The liver is of average size with two hypodense focal lesions seen in segment VII the largest measures 7x8 mm, they are inconspicuous in the delayed phase.
- Focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys, adrenals, aorta, I. V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes
- Bilateral simple ovarian cysts, with mild pelvic collection.
- **The revised bone window revealed:**
 1. Left iliac bone subchondral sclerotic oval lesion measuring 20x20 mm

likely bone island.

2. No CT evidence of vertebral collapse or extra-osseous soft tissue component.

Opinion:

- Right Breast UOQ hyperdense lesions as described.
- Middle lobe pulmonary nodule likely non specific for follow up.
- Hepatic segment VII small hypodense lesions for better evaluation by dynamic contrast enhanced MRI study.
- Left iliac crest sclerotic lesion likely bone island.

Reported By,

Dr. Kareem Moussa, MD
Dr. Yasmine Gamal, MSc

06-Sep-2021 03:22 PM

Patient's Name:

Fatima Mohammed Anwar Mohammed Ahmed

Date:

2026-07-22

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Status post right MRM & ALAND, routine follow up.

Findings:

Left breast:

- Scattered fibro-glandular densities (ACR b)
- UOQ circumscribed oval equal density mass lesion with calcific foci.
- No speculations, asymmetry, distortion or suspicious microcalcifications.
- Ultrasonnd demonstrated 3 clock; circumscribed oval parallel homogenous hypoechoic benign looking solid mass lesion with absent vascularity and posterior characters, measuring 8 X 2 mm
- No cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs

Right mastectomy bed:

- Clear mastectomy bed with no masses or fluid collections.
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

Conclusion:

Left breast (BIRADS 2): Small UOQ adenoma.

Clear right mastectomy bed and axilla.

Recommendation:

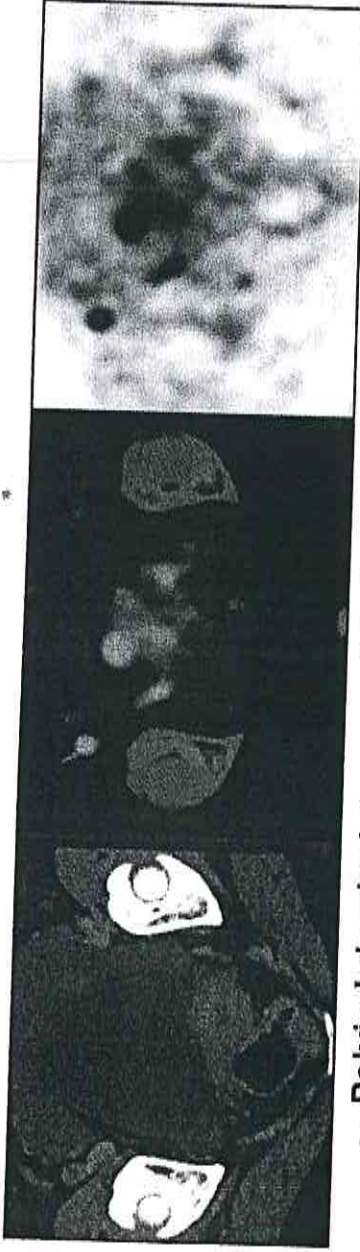
For annual follow up.

Best Regards,
Dr. Amal Hegazi, M.Sc.

BIRADS 2: Scattered breast lesions according to the American College of Radiology used for breast assessment combined

with breast cancer risk	
Category	Estimated Cancer Risk
BIRADS 1: No suspicious findings	0 %
BIRADS 2: Scattered breast lesions	0 %
BIRADS 3: Scattered breast lesions	0 %
BIRADS 4: Scattered breast lesions	0 %
BIRADS 5: Scattered breast lesions	0 %
BIRADS 6: Scattered breast lesions	0 %
BIRADS 7: Scattered breast lesions	0 %
BIRADS 8: Scattered breast lesions	0 %
BIRADS 9: Scattered breast lesions	0 %
BIRADS 10: Scattered breast lesions	0 %
BIRADS 11: Scattered breast lesions	0 %
BIRADS 12: Scattered breast lesions	0 %
BIRADS 13: Scattered breast lesions	0 %
BIRADS 14: Scattered breast lesions	0 %
BIRADS 15: Scattered breast lesions	0 %
BIRADS 16: Scattered breast lesions	0 %
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BIRADS 94: Scattered breast lesions	0 %
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BIRADS 96: Scattered breast lesions	0 %
BIRADS 97: Scattered breast lesions	0 %
BIRADS 98: Scattered breast lesions	0 %
BIRADS 99: Scattered breast lesions	0 %
BIRADS 100: Scattered breast lesions	0 %

• Lymph nodes:



- o Pelvi-abdominal: increased FDG uptake within left gastric, celiac, porto-caval, retro-caval, aorto-caval, left para-aortic, mesenteric, pre-sacral, para-rectal, left common, bilateral internal and external iliac lymph nodes, a left para-aortic node with SUVmax 15.7 and measures 2.4 x 2.2 cm, left common node with SUVmax 12.6 and measures 1 cm and right external iliac node with SUVmax 10.2 and measures 1.4 cm.



- o Mediastinal: increased FDG uptake prevascular, retro-sternal, bilateral internal mammary, epiphrenic and cardiophrenic lymph nodes, prevascular node with SUVmax 8.0 and measures 1.2 cm and right internal node with SUVmax 11.4 and measures 0.8 cm.

Patient's Name:

Fatima Mohammed Anwar Mohammed Ahmed

Date:

2026-07-22

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Status post right MRM & ALAND, routine follow up.

Findings:

Left breast:

- Scattered fibro-glandular densities (ACR b)
- UOQ circumscribed oval equal density mass lesion with calcific foci.
- No speculations, asymmetry, distortion or suspicious microcalcifications.
- Ultrasonnd demonstrated 3 clock circumscribed oval parallel homogenous hypoechoic benign looking solid mass lesion with absent vascularity and posterior characters, measuring 8 X 2 mm
- No cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs

Right mastectomy bed:

- Clear mastectomy bed with no masses or fluid collections.
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

Conclusion:

Left breast (BIRADS 2): Small UOQ adenoma.

Clear right mastectomy bed and axilla.

Recommendation:

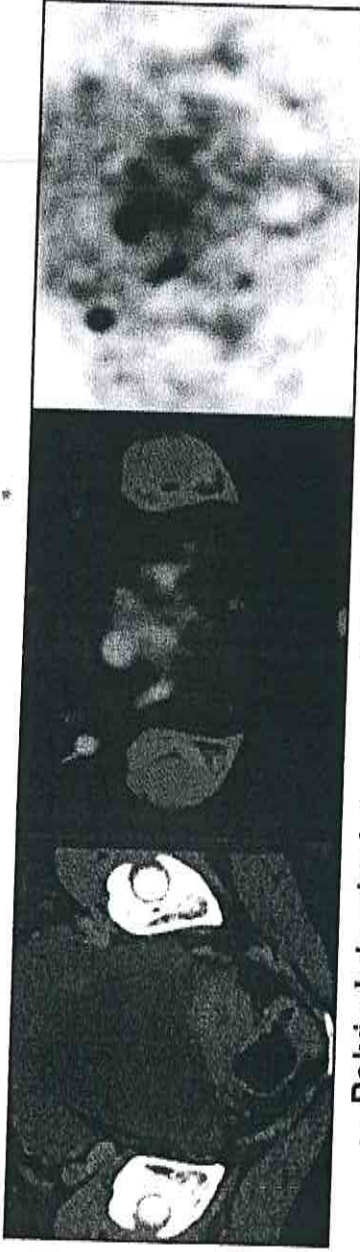
For annual follow up.

Best Regards,
Dr. Amal Hegazi, M.Sc.

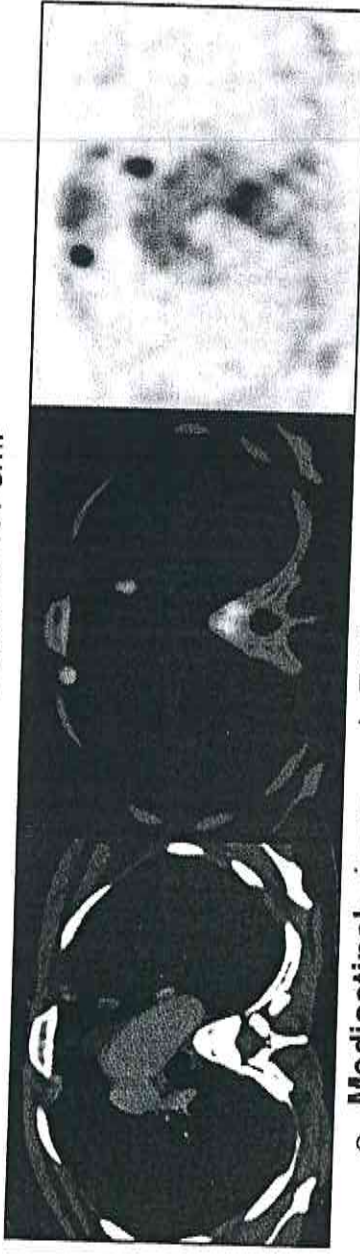
BIRADS 2: Scattered breast lesions according to the American College of Radiology used for breast assessment combined

with breast cancer risk	
Estimated Cancer Risk	
Category 1: Scattered breast lesions	0 %
BIRADS 2: Scattered breast lesions	0 %
BIRADS 3: Scattered breast lesions	2 %
BIRADS 4: Scattered breast lesions	50 %
BIRADS 5: Scattered breast lesions	50 %
BIRADS 6: Scattered breast lesions	50 %
BIRADS 7: Scattered breast lesions	50 %
BIRADS 8: Scattered breast lesions	50 %
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BIRADS 48: Scattered breast lesions	50 %
BIRADS 49: Scattered breast lesions	50 %
BIRADS 50: Scattered breast lesions	50 %

• Lymph nodes:



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Patient's Name:

Fatima Mohammed Anwar Mohammed Ahmed

Date:

2026-07-22

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt



Left Digital soft tissue mammography with high resolution sonography revealed:

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Right mastectomy bed:

- Clear mastectomy bed with no masses or fluid collections.
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Conclusion:

Left breast (BIRADS 2): Small UOQ adenoma.

Clear right mastectomy bed and axilla.

Recommendation:

For annual follow up.

Best Regards,
Dr. Amal Hegazi, M.Sc.

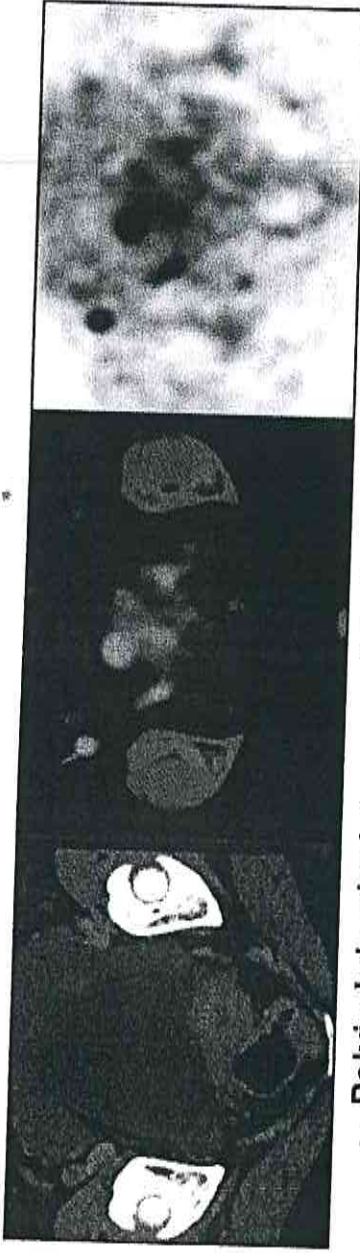
BIRADS 2: Scattered breast lesions according to the American College of Radiology used for breast assessment combined

with breast cancer risk	
Category	Estimated Cancer Risk
BIRADS 1: No suspicious findings	0 %
BIRADS 2: Scattered breast lesions	0 %
BIRADS 3: Scattered breast lesions	0 %
BIRADS 4: Scattered breast lesions	0 %
BIRADS 5: Scattered breast lesions	0 %
BIRADS 6: Scattered breast lesions	0 %
BIRADS 7: Scattered breast lesions	0 %
BIRADS 8: Scattered breast lesions	0 %
BIRADS 9: Scattered breast lesions	0 %
BIRADS 10: Scattered breast lesions	0 %
BIRADS 11: Scattered breast lesions	0 %
BIRADS 12: Scattered breast lesions	0 %
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BIRADS 99: Scattered breast lesions	0 %
BIRADS 100: Scattered breast lesions	0 %



S C A N

• Lymph nodes:



- Pelvi-abdominal: increased FDG uptake within left gastric, celiac, porto-caval, retro-caval, aorto-caval, left para-aortic, mesenteric, pre-sacral, para-rectal, left common, bilateral internal and external iliac lymph nodes, a left para-aortic node with SUVmax 15.7 and measures 2.4 x 2.2 cm, left common node with SUVmax 12.6 and measures 1 cm and right external iliac node with SUVmax 10.2 and measures 1.4 cm.



- Mediastinal: increased FDG uptake prevascular, retro-sternal, bilateral internal mammary, epiphrenic and cardiophrenic lymph nodes, prevascular node with SUVmax 8.0 and measures 1.2 cm and right internal node with SUVmax 11.4 and measures 0.8 cm.